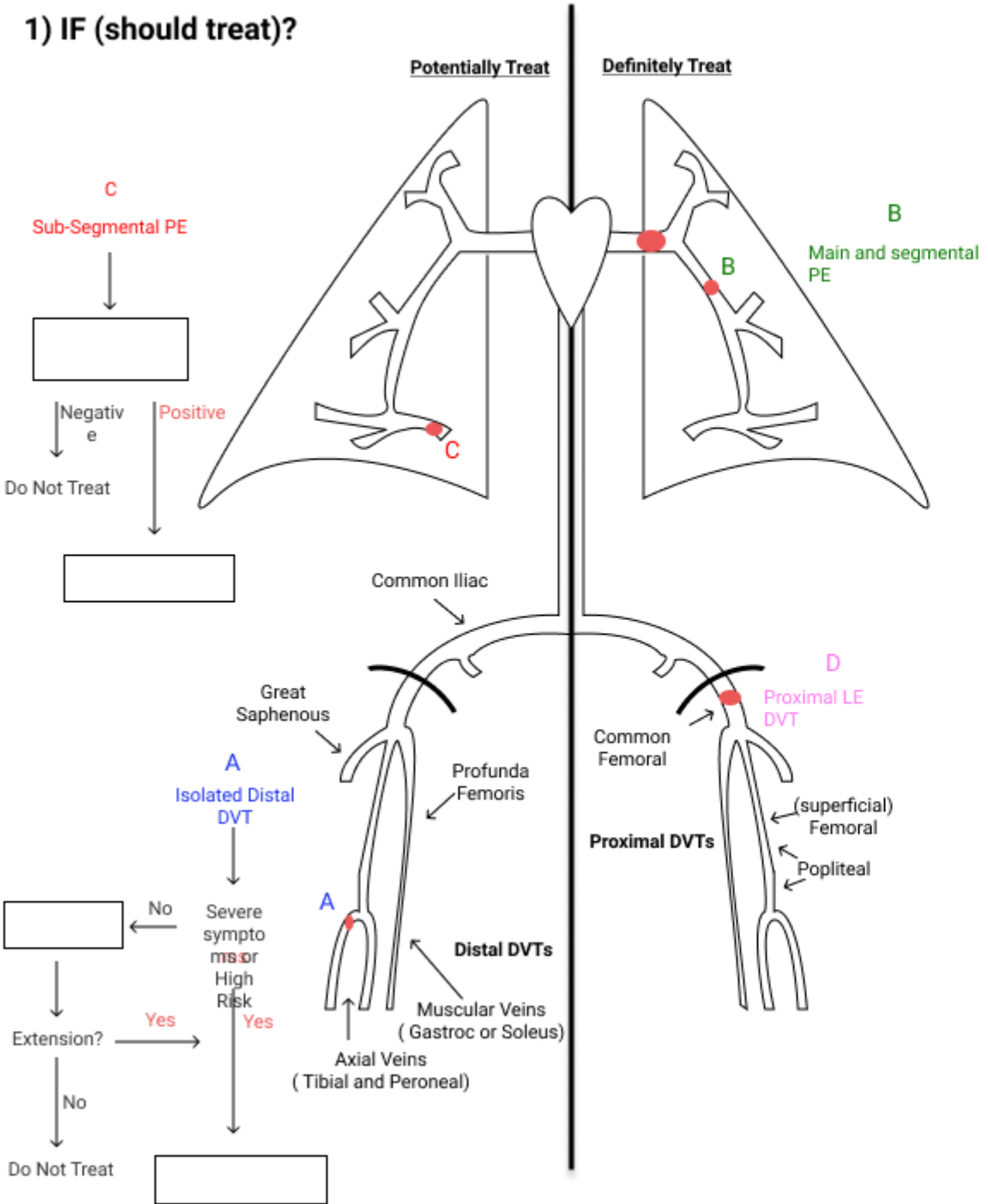
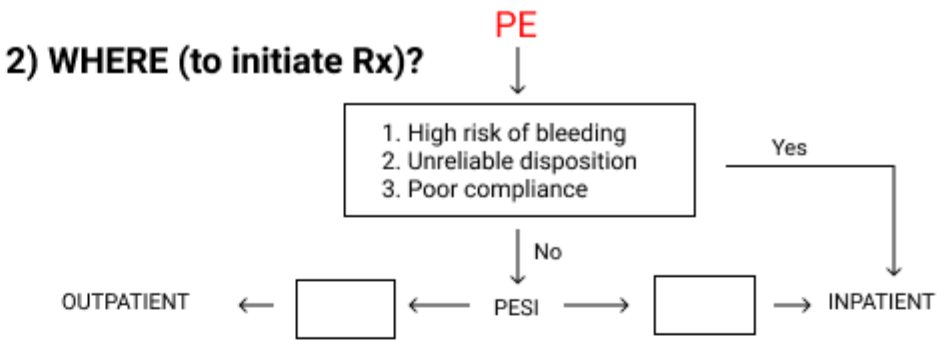


1) IF (should treat)?



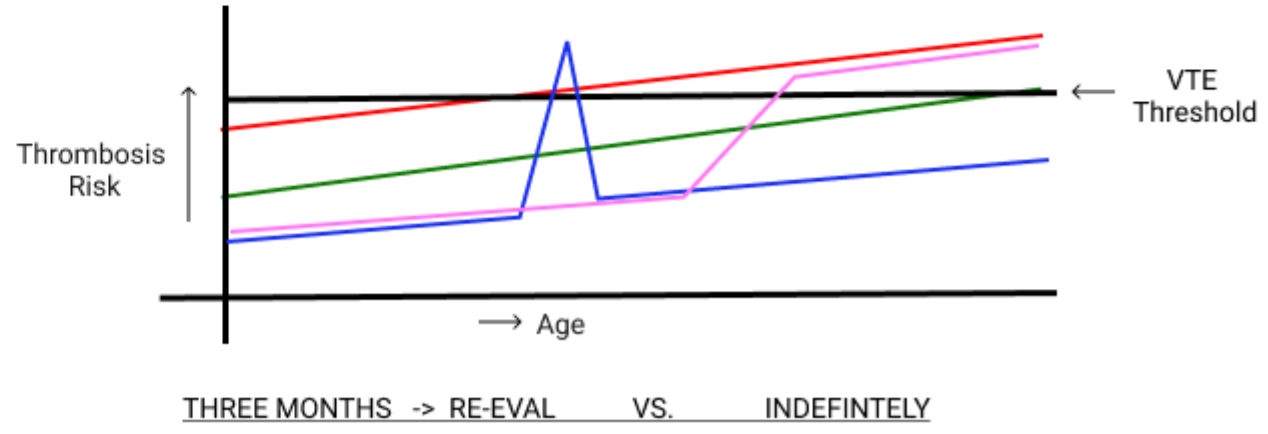
2) WHERE (to initiate Rx)?



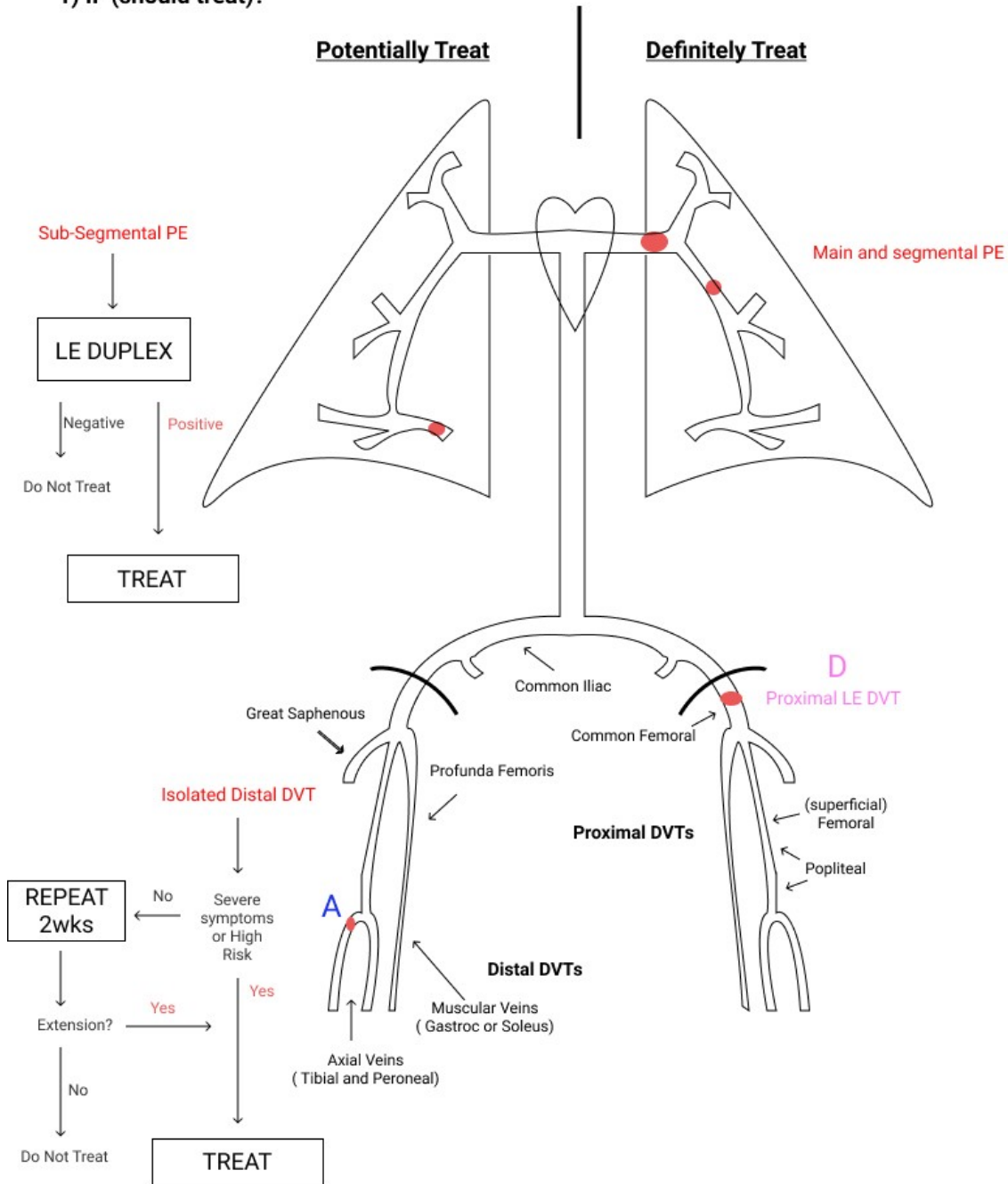
3) WHICH (agent)?

|          | Bridge? | Primary Use? |
|----------|---------|--------------|
| LMWH     |         |              |
| DOAC     |         |              |
| Warfarin |         |              |

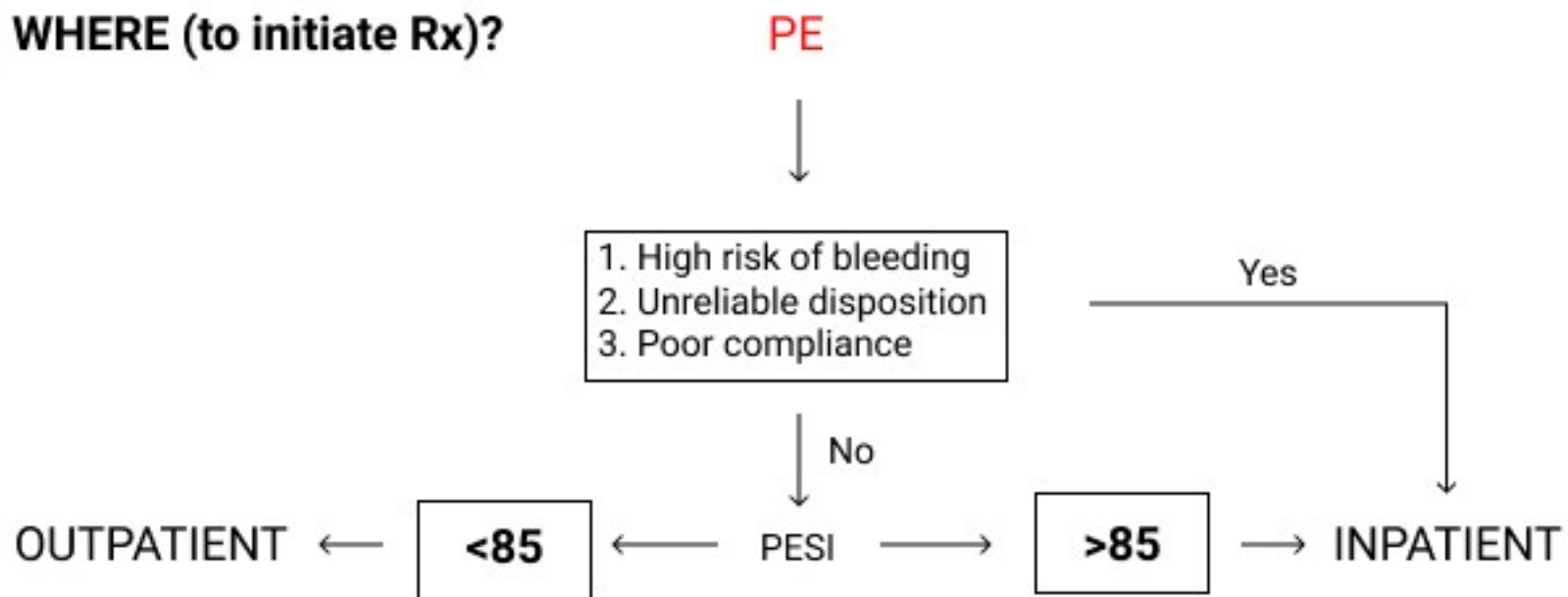
4) HOW LONG?



1) IF (should treat)?



## 2) WHERE (to initiate Rx)?



### 3) WHICH (agent)?

|          | Bridge?                                               | Common Use? |
|----------|-------------------------------------------------------|-------------|
| LMWH     | NO                                                    | Bridge      |
| DOAC     | NO<br>(Dabigatran & Edoxaban<br>require 5d "lead-in") | First-Line  |
| Warfarin | YES                                                   | GFR <25     |

#### 4) HOW LONG (to treat)?

THREE MONTHS → RE-EVAL

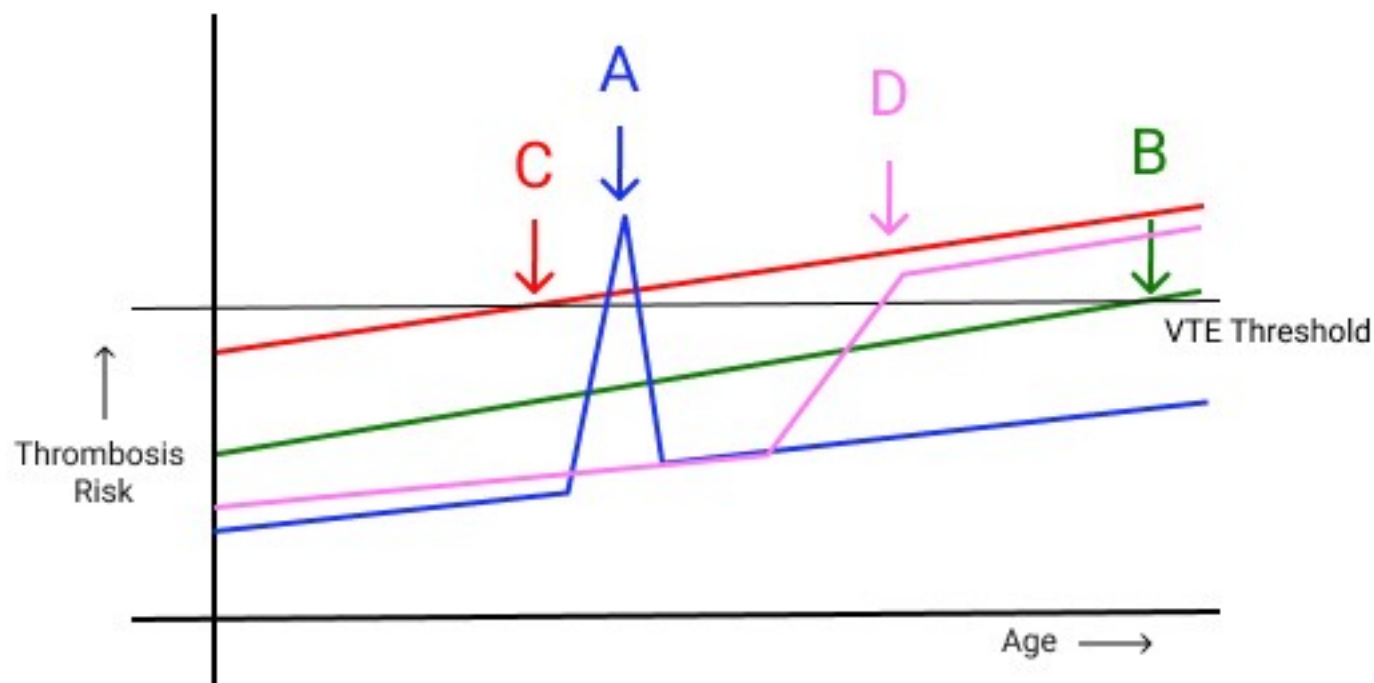
INDEFINITELY

**A.** Provoked  
(cause is reversible)

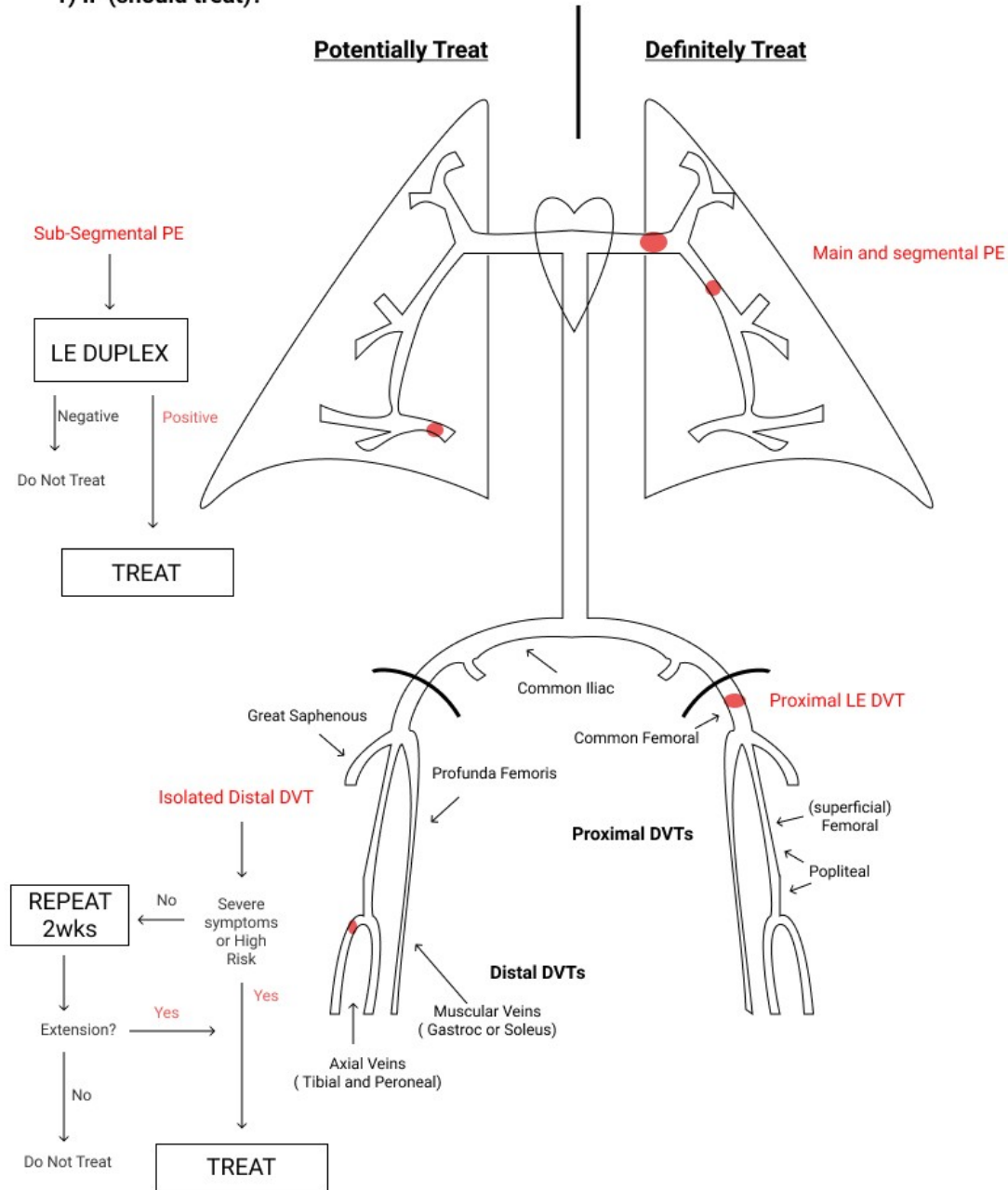
**B.** Unprovoked but  
HGH bleed risk

**C.** Unprovoked  
(NOT high bleed risk)

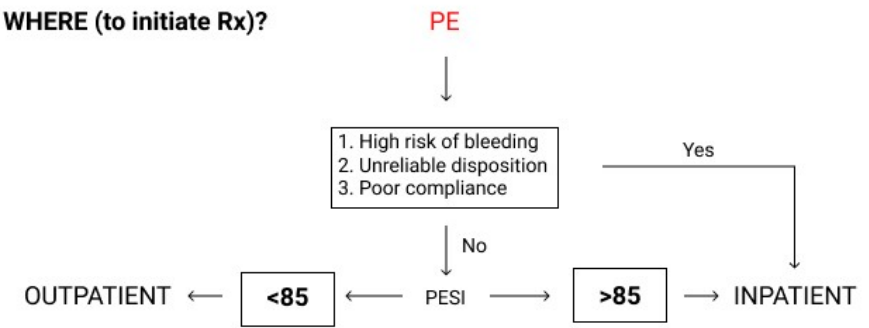
**D.** Provoked  
(Irreversible Cause)



## 1) IF (should treat)?



## 2) WHERE (to initiate Rx)?



## 3) WHICH (agent)?

|          | Bridge?                                            | Common Use? |
|----------|----------------------------------------------------|-------------|
| LMWH     | NO                                                 | Bridge      |
| DOAC     | NO<br>(Dabigatran & Edoxaban require 5d "lead-in") | First-Line  |
| Warfarin | YES                                                | GFR <25     |

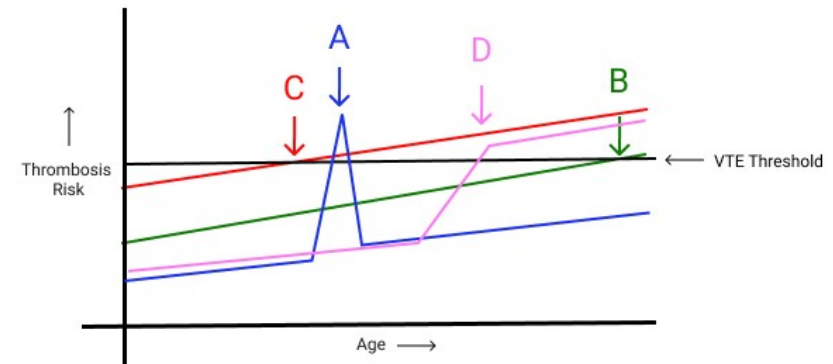
## 4) HOW LONG?

**THREE MONTHS → RE-EVAL**

- A. Provoked (cause is reversible)**
- B. Unprovoked but HGH bleed risk**

**INDEFINITELY**

- C. Unprovoked (NOT high bleed risk)**
- D. Provoked (Irreversible Cause)**



# 2020–25 Update — Agent Choice, Duration & Special Cases

## **Cancer-associated VTE — DOACs are now first-line over LMWH:**

Apixaban, edoxaban, or rivaroxaban (CARAVAGGIO 2020: apixaban noninferior to dalteparin, no excess major bleeding; ASH 2021, ASCO, ITAC, CHEST 2021).

Apixaban or LMWH preferred for luminal GI malignancies (higher GI bleeding with rivaroxaban/edoxaban). LMWH still favored with poor oral absorption or major drug interactions.

## **Extended (>3–6 mo) therapy — reduced-dose DOAC lowers bleeding:**

Apixaban 2.5 mg BID (AMPLIFY-EXT) or rivaroxaban 10 mg daily (EINSTEIN-CHOICE) for the extended phase after the initial treatment period.

## **Special cases:**

Antiphospholipid syndrome (esp. triple-positive): use warfarin, NOT a DOAC (TRAPS — excess arterial events with rivaroxaban).

Isolated subsegmental PE without proximal DVT & low recurrence risk: surveillance is an option over anticoagulation (CHEST 2021) — matches the board's SSPE → duplex pathway.

**Standing framework (IF / WHERE / WHICH / HOW LONG) unchanged: PESI <85 → outpatient; DOAC first-line; unprovoked without high bleed risk → indefinite.**

# Take-Home Points

IF: not every clot is treated — isolated distal DVT (low risk, asymptomatic) can be surveilled with repeat duplex in 1–2 weeks; isolated subsegmental PE without proximal DVT may be surveilled.

WHERE: an uncomplicated PE with PESI <85 (or sPESI 0) can be managed as an outpatient; admit for high bleeding risk, unreliable disposition, or poor compliance.

WHICH: DOACs are first-line for stable VTE, including most cancer-associated VTE; LMWH/apixaban preferred for luminal GI cancers; warfarin for antiphospholipid syndrome and CrCl <25–30.

HOW LONG: provoked by a reversible cause → ~3 months; unprovoked or an irreversible/persistent cause without high bleed risk → indefinite anticoagulation (consider reduced-dose DOAC for the extended phase).



# Attribution & Optimization Note

Adapted from TeachIM.org — “Management of Venous Thromboembolism (VTE)” (Meagan Wong MD, Brandon Fainstad MD; expert review David Garcia MD). Published 2020.

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Deck note: TeachIM publishes this as a non-interactive series of whiteboard board-set-up images (no editable text runs) intended to be reproduced on a whiteboard. No surgical text edits were possible or needed — the IF/WHERE/WHICH/HOW-LONG framework matches current guidance.

Added context (source-cited): cancer-VTE DOAC-first (CARAVAGGIO 2020; ASH 2021/CHEST 2021), reduced-dose extended DOAC (AMPLIFY-EXT, EINSTEIN-CHOICE), APS→warfarin (TRAPS), subsegmental-PE surveillance.

Companion files: VTE-Learners-Handout.pdf (4 cases), VTE-Teachers-Guide.pdf. Educational use only; not medical advice — have content clinician-reviewed before teaching.